



SOFTWARE. AI VISION. ROBOTICS.

AI and robotics for healthcare.

Connecting what happens in a hospital to
information its teams can act on.

STERIX is developing intelligent systems for hospital
operations, starting with supply visibility.

OUR STARTING POINT

EAGLE EYE

Prototype built.
Hospital case study underway.



CONCEPT ILLUSTRATION
Not the Eagle Eye prototype

START WITH A SPECIFIC WORKFLOW

THE GAP WE'RE TARGETING.

A supply check should lead to a clear next step, not just another observation.

- 01

SEE WHAT IS THERE

Check the presence of gloves, wipes and briefs. Establish what camera observations can reliably show.
- 02

KNOW WHO ACTS

Make an exception visible to the appropriate team, with a clear owner and an agreed response.
- 03

KNOW WHAT HAPPENED

Connect the finding to staff review and, in the planned workflow, a record of completed action.



THE QUESTION WE'RE TESTING

Can camera-derived evidence reduce checking effort and improve exception follow-through?

CONCEPT ILLUSTRATION / NOT PROTOTYPE HARDWARE

BUILT AROUND EXISTING OPERATIONS

Hospitals already use inventory software, barcodes, RFID and replenishment systems. Our intended buyers are supply-chain and operations teams, working with IT and clinical staff.

Workflow hypothesis, not a proven market-wide gap. Established approaches: GS1 Healthcare standards; PAR Excellence inventory automation. See source notes.

ADOPTION CONTEXT, NOT OUR ADDRESSABLE MARKET

WHY NOW: ROBOTICS AT SCALE.

Robotics is moving from technical possibility to repeated use. Healthcare adoption still has to be earned, workflow by workflow.



3.15_M

ROBOT-ASSISTED PROCEDURES

Procedures performed worldwide in 2025 using Intuitive's da Vinci surgical systems.

Intuitive Surgical / 2025 Form 10-K



\$26.3_B

MEDICAL ROBOTICS MARKET

IMARC's global 2025 estimate, with a \$97.8B forecast for 2034 across medical robotic systems.

Third-party market estimate and forecast



4.66_M

INDUSTRIAL ROBOTS IN USE

Operational stock worldwide in 2024, across industries. Evidence of broader industrial adoption.

International Federation of Robotics / 2025

OUR RESPONSE

Take one operational application through hospital evaluation, prove its usefulness, then build toward repeatable deployment.

[Intuitive 2025 Form 10-K, p. 68](#) · [IMARC: Medical Robotic Systems Market](#) · [IFR: World Robotics 2025](#)

Different categories and measurement periods. These figures are context, not STERIX revenue, demand or TAM.

PLANNED ARCHITECTURE / UNDER DEVELOPMENT

THE SOFTWARE THAT CONNECTS THE WORK.

A shared record of evidence, operating rules, exceptions, approvals and what happened next.

CURRENT APPLICATION

PRESENCE AND REPORTING

Eagle Eye starts with supply observations and daily reports for staff review.

PLANNED WORKFLOWS

RECONCILIATION AND CLOSURE

Connect inventory exceptions, findings or recalls to owners, deadlines, escalation and reviewed closure.

FUTURE ROBOT OPERATIONS

AUTHORIZED ACTION

Extend the same record to robot permissions, stop states, maintenance and staff handoffs.



CONCEPT INTERFACE / ILLUSTRATIVE DATA

Evidence, ownership and review history in one intended operating view. Not a live product screenshot or a deployed integration.

01 **OBSERVE**

→ 02

**IDENTIFY WHAT
NEEDS ATTENTION**

→

03 **ASSIGN AN OWNER**

→

04 **RECORD COMPLETION**

Intended architecture from STERIX platform and strategy materials. Human accountability remains with staff. This is not a verified, deployed end-to-end platform.

CURRENT PROOF POINT

EAGLE EYE: BUILT AND UNDER EVALUATION.

The first application is deliberately narrow: supply presence, daily reports and staff review.

WHAT EXISTS

A built prototype.
A hospital case study underway.

Founder-reported status. The current workflow tracks gloves, wipes and briefs. Camera configuration and integration are being assessed.

 The pitch illustrations show a design direction, not photographs or exact renderings of the hospital prototype.

WHAT WE'RE TESTING

- 01 Agreement with staff spot checks
- 02 Missed and incorrect presence reports
- 03 Completeness of daily reports
- 04 Staff effort to review and respond
- 05 Installation and integration requirements

Agree acceptance thresholds before evaluation. No performance results are claimed here.



Source: founder status and September 14, 2026 integration/demo meeting summary. Proposed evaluation measures, not achieved results.

A SHARED SOFTWARE APPROACH / A GATED HARDWARE ROADMAP

WHAT WE'RE BUILDING NEXT.

Different hospital tasks. A common ambition: useful evidence, human review and accountable action.



MOBILE OBSERVATION / CONCEPT

SCOUT

Mobile inventory and camera observation, connected to records that staff can review.



INSPECTION + HANDLING / CONCEPT

STERISIGHT + TRAYGUARD

Inspection records and assisted handling. Staff retain final release responsibility.



SCREENING / CONCEPT

HEALTHPOD

Selected screening measurements organized for clinician review, not autonomous diagnosis.



LONG-TERM / RESEARCH DIRECTION

FUTURE SURGICAL RESEARCH

A separate research horizon, not a built product or a commitment to clinical deployment.

All images are concept illustrations; depicted shapes are not exact prototypes. Shared integration is an ambition, not a validated capability. Concepts are subject to feasibility, safety and applicable regulatory work.

PROPOSED COMMERCIAL MODEL

FROM EVALUATION TO RECURRING REVENUE.

Sell a defined operational outcome first. Earn the right to become a recurring part of the workflow.

01

PAID EVALUATION

A scoped workflow, agreed acceptance criteria and a documented path to a paid site agreement.

02

RECURRING SOFTWARE

Annual software and support per paid site, with responsibilities and service scope made explicit.

03

SEPARATE DEPLOYMENT

Hardware, installation and integration priced separately rather than hidden in software economics.

ILLUSTRATIVE ANNUAL PRICE / SITE

\$60K

5 PAID SITES

\$300K

ANNUAL RECURRING REVENUE

10 PAID SITES

\$600K

ANNUAL RECURRING REVENUE

20 PAID SITES

\$1.2M

ANNUAL RECURRING REVENUE

WHAT MUST BE PROVEN

Budget ownership, procurement and security approval, willingness to pay, and the full cost of delivering and supporting each site.

Pricing is an unvalidated planning assumption. Site counts are arithmetic scenarios, not pipeline, contracted customers or a revenue forecast. Hardware and project fees excluded from ARR.

COLLECTIVE EXPERIENCE

EXPERIENCE TO BUILD AND DEPLOY.

Hospital technology needs more than a working device. It needs a workflow people can use, install, support and buy.



HOSPITAL OPERATIONS

Translate day-to-day supply and care processes into practical requirements and meaningful evaluation criteria.

WORKFLOW UNDERSTANDING



INDUSTRIAL AUTOMATION

Connect hardware, controls and installation requirements to a system that can be tested and maintained.

ENGINEERING + INSTALLATION



PRODUCT DESIGN

Make observations and exceptions understandable, with clear interaction, review and handoff responsibilities.

USABILITY + HUMAN REVIEW



BUSINESS DEVELOPMENT

Define the buyer, paid scope, procurement path and partnerships needed to turn evaluation into adoption.

COMMERCIAL EXECUTION

THE OPERATING PRINCIPLE

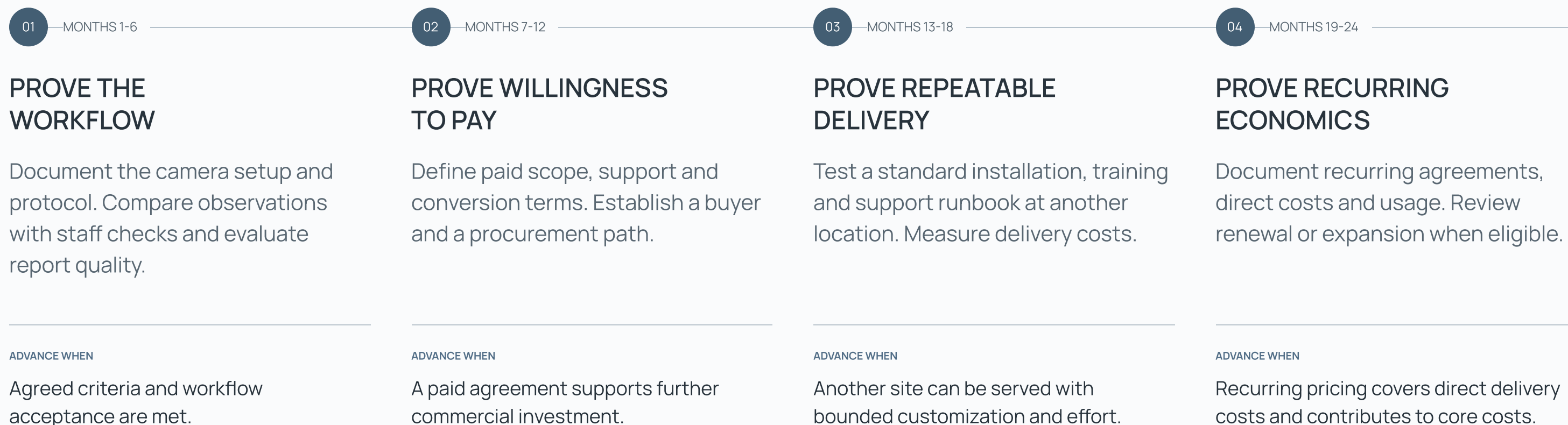
Build with the people who do the work.
Advance on documented evidence.

Founder-described collective capability areas. No individual credentials, named advisers or quantified track record are asserted on this page.

PROPOSED PROGRAM / MONTHS FROM FUNDING

FOUR MILESTONES FOR THE NEXT 24 MONTHS.

Each stage answers a different investment question. Progress depends on evidence, not the calendar alone.



Selected concept feasibility may begin after Month 12, subject to the operating gates and the existing budget. Future surgical development and trials remain separate.

Proposed stages, not promised results or dates of regulatory clearance. Detailed evaluation gates and cost categories are in the supporting notes.

PROPOSED INVESTMENT / 24-MONTH PROGRAM

VALIDATE. DEPLOY. EXPAND.

\$6M

Fund the evidence, software and delivery capability needed to move from evaluation to repeatable adoption.

\$4.8M	80.0%	TEAM, SOFTWARE + OPERATIONS Build the workflow, support hospital evaluation and establish repeatable delivery.
\$0.5M	8.3%	EQUIPMENT, INTEGRATION + EVALUATION Test the camera configuration, installation requirements and external evaluation needs.
\$0.7M	11.7%	RESERVE Maintain flexibility as technical and deployment assumptions are tested.



PITCH.STERIX.US

Up to \$0.5M for selected concept feasibility is **included within the \$5.3M planned spend**, gated after Month 12. It is not additional funding. Future surgical development and trials are separate.

PROVE ONE WORKFLOW. BUILD A FOUNDATION FOR WHAT COMES NEXT.

Proposed use of funds and management planning assumptions. Program funding does not depend on assumed customer receipts. Background: concept illustration, not a STERIX-owned facility.